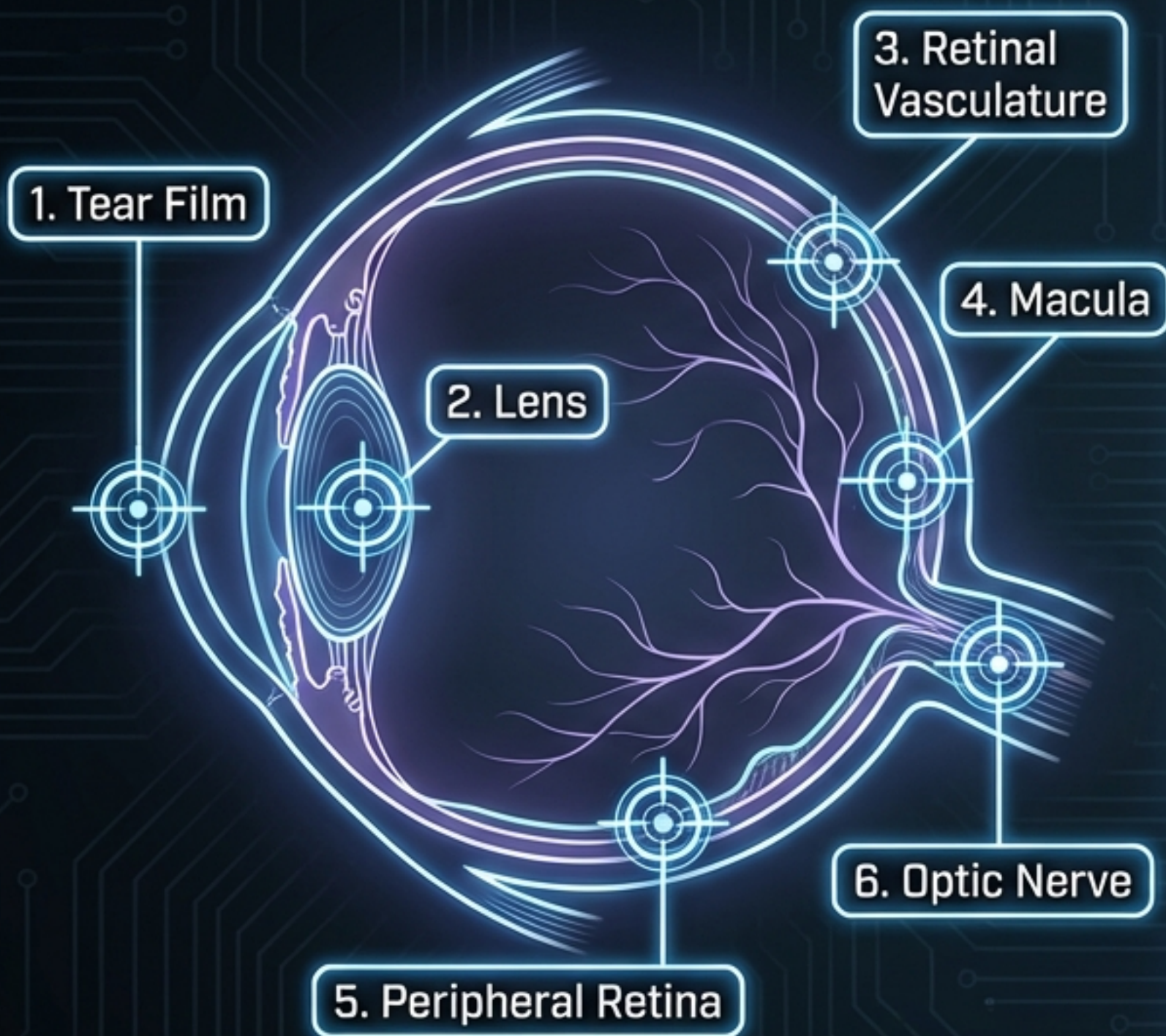




The Spatial Diagnostic Journey

Module 4: Rapid Clinical Differentiation of Common Eye Diseases

TARGET AUDIENCE: Allied Ophthalmic Personnel | **INTENT:** Clinical Reference Guide | **VIEW:** Diagnostic HUD



DIAGNOSTICS REFERENCE MANUAL

CAUSES

[Pathology Timeline]



SYMPTOMS

[Patient POV Simulation]



DIAGNOSTICS

[Clinical Targets]



TREATMENTS

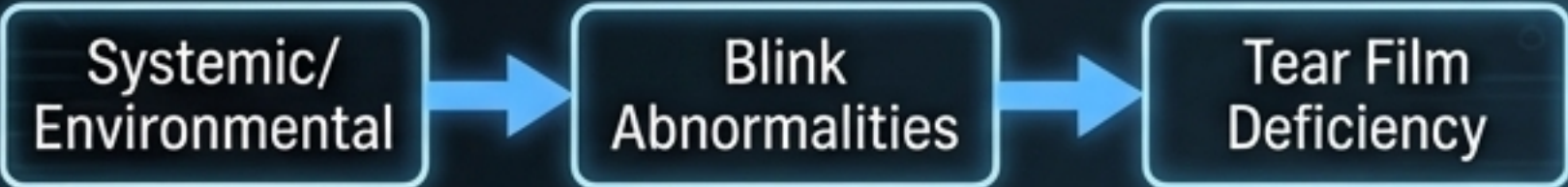
[Intervention Hierarchy]



The fragile ecosystem of the eye means localized anatomical damage creates highly specific visual field defects. We will travel front-to-back, isolating six distinct pathologies.

DIAGNOSTICS REFERENCE MANUAL

CAUSES

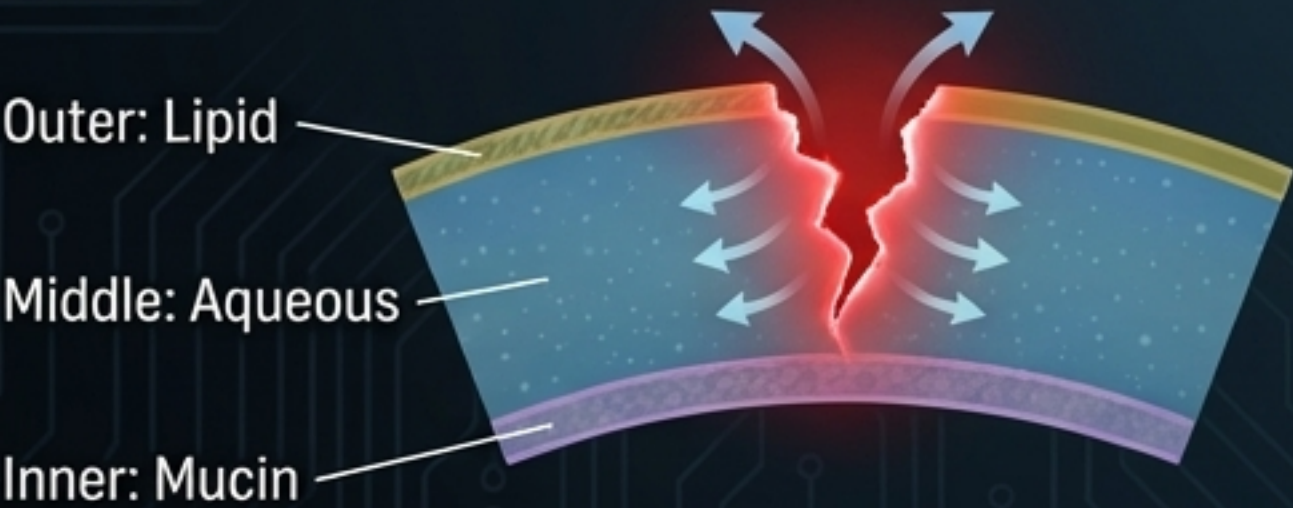


SYMPTOMS



DIAGNOSTICS

Schirmer Test,
Tear Break-Up
Time (TBUT),
Rose Bengal



TREATMENTS



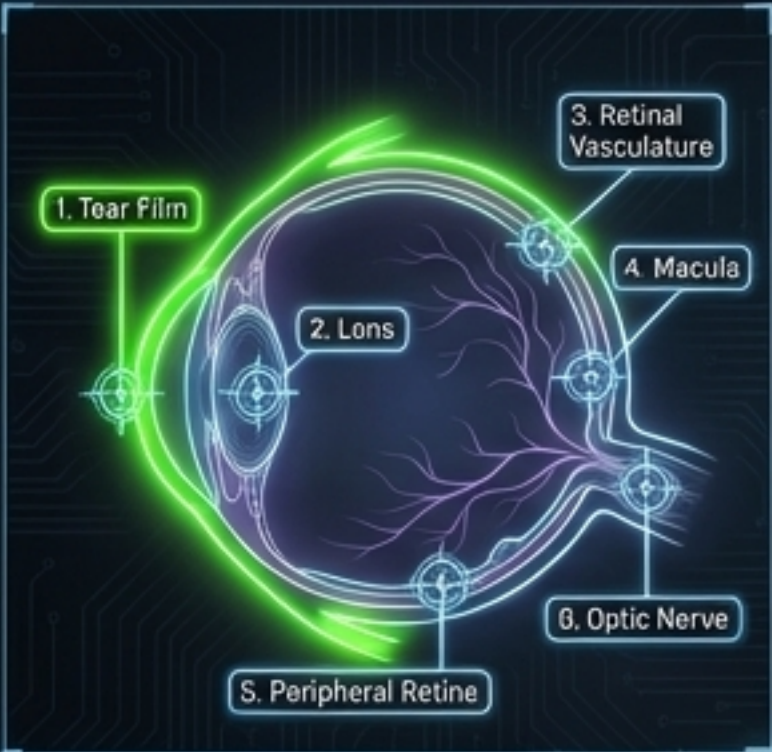
Artificial tears,
Restasis



Nutritional
supplements



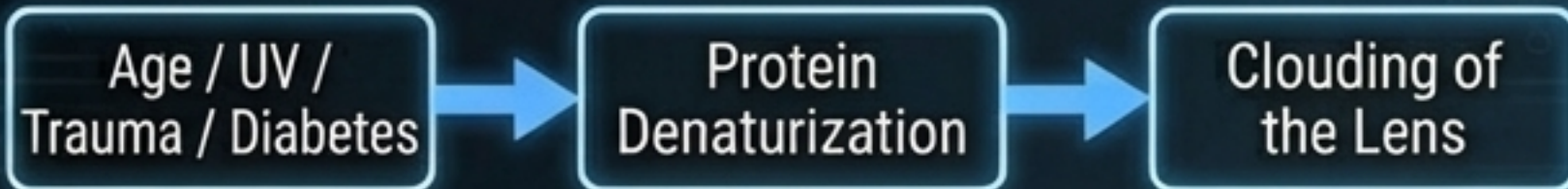
Meibomian expression,
Punctal occlusion



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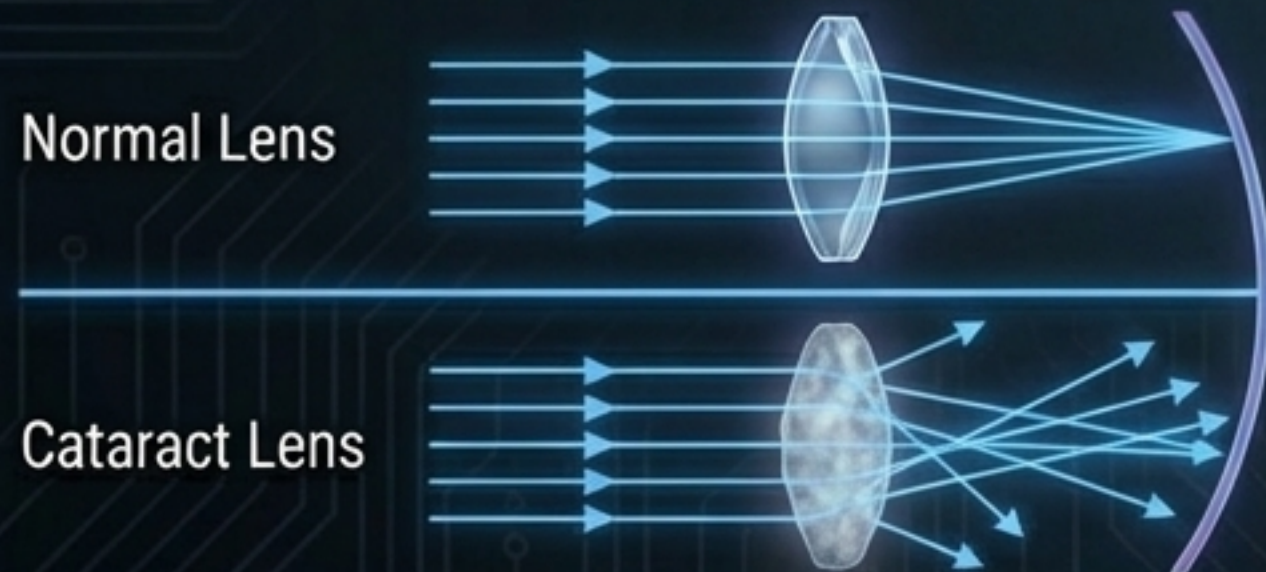
CAUSES



SYMPTOMS

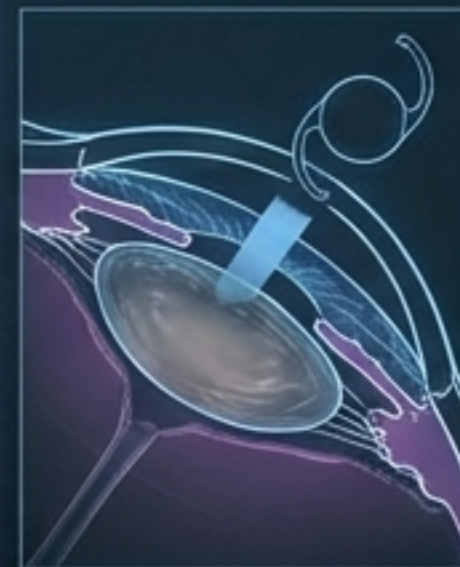


DIAGNOSTICS

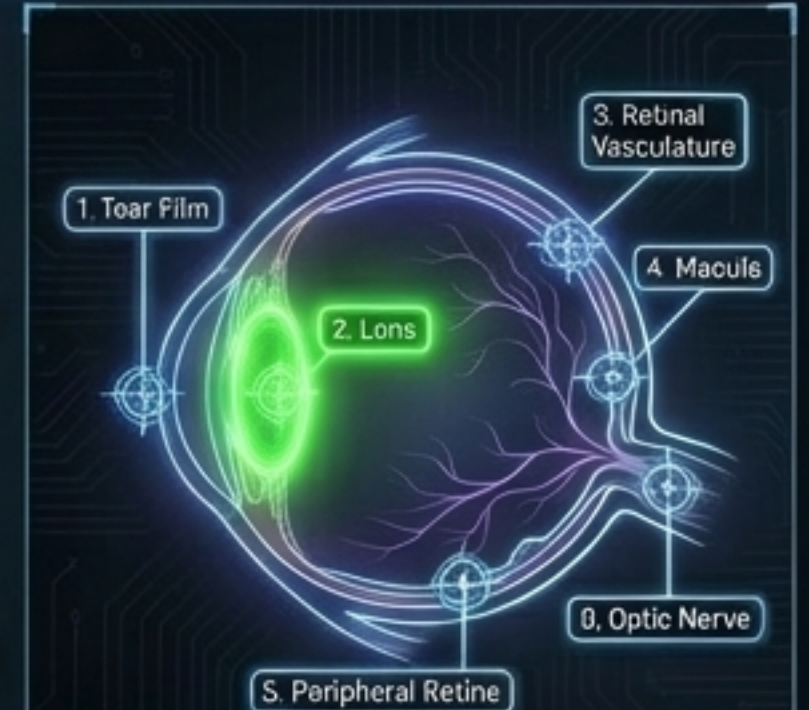


Slit-lamp evaluation,
Potential Acuity
Meter (PAM),
Glare testing
(BAT)

TREATMENTS



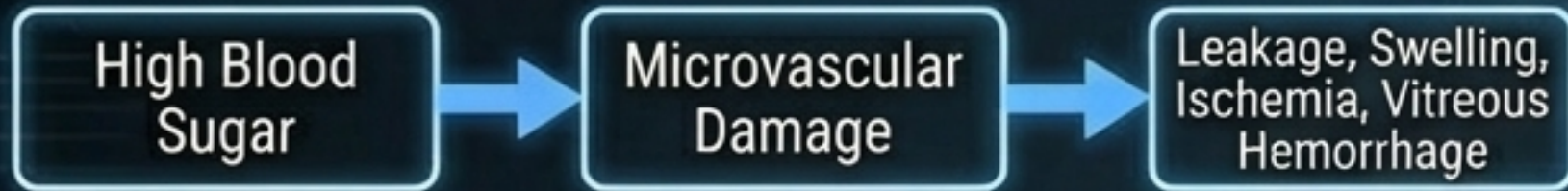
YAG capsulotomy
(post-surgery clearing)



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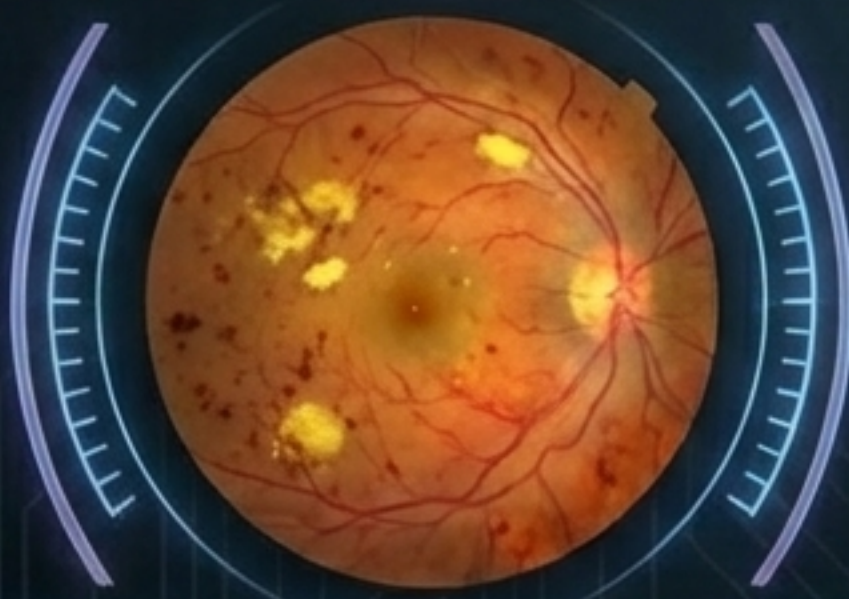
CAUSES



SYMPTOMS



DIAGNOSTICS



OCT,
Fluorescein
Angiography,
Fundus
Photography

TREATMENTS



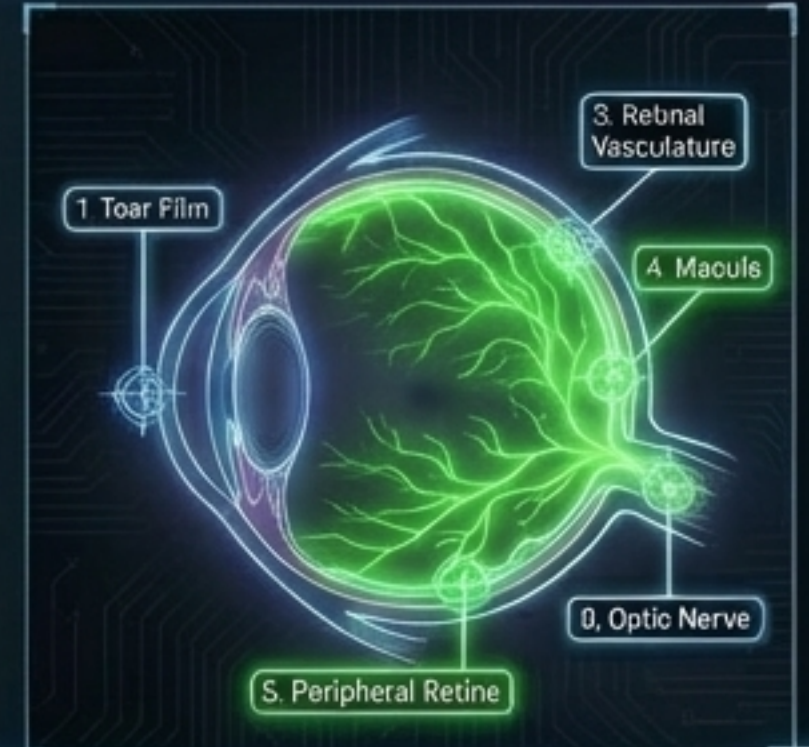
Anti-VEGF injections



Systemic blood
sugar control



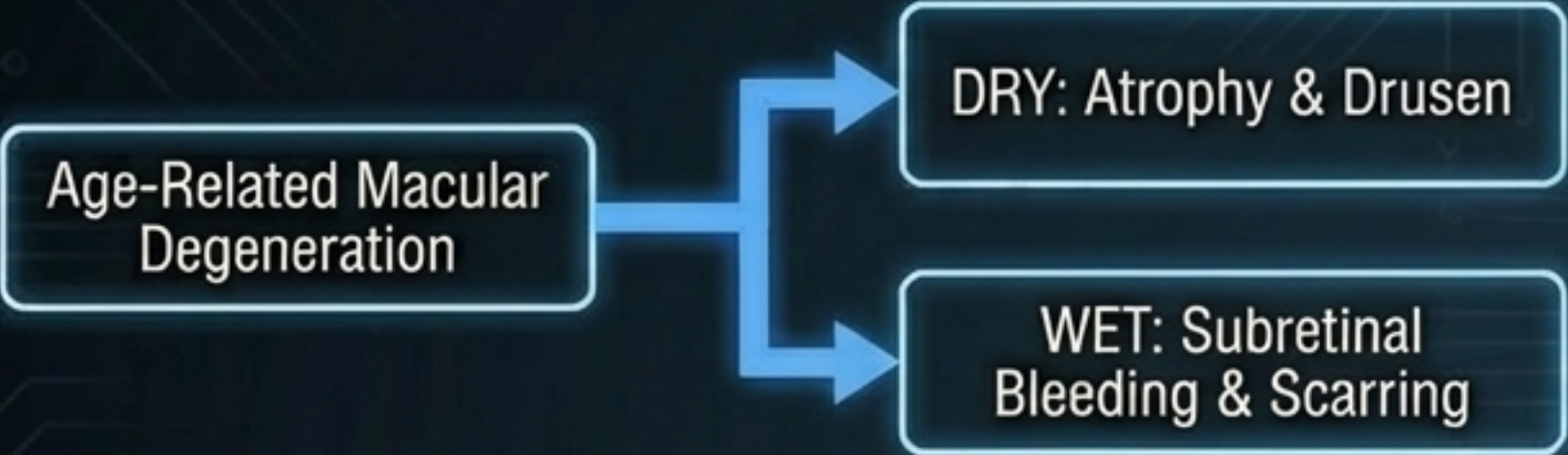
Panretinal
photocoagulation



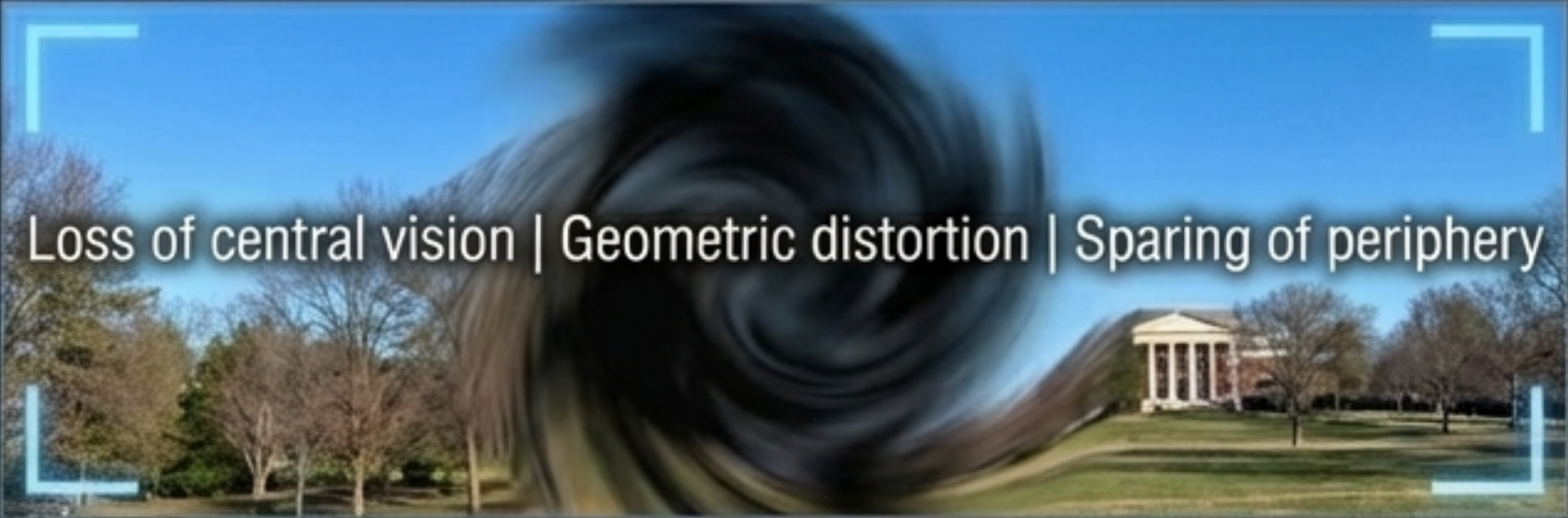
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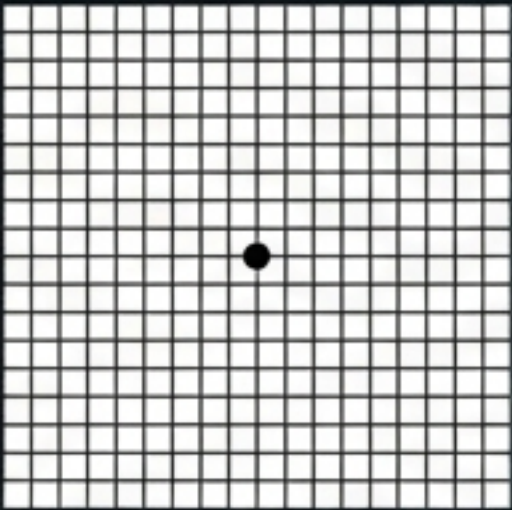
CAUSES



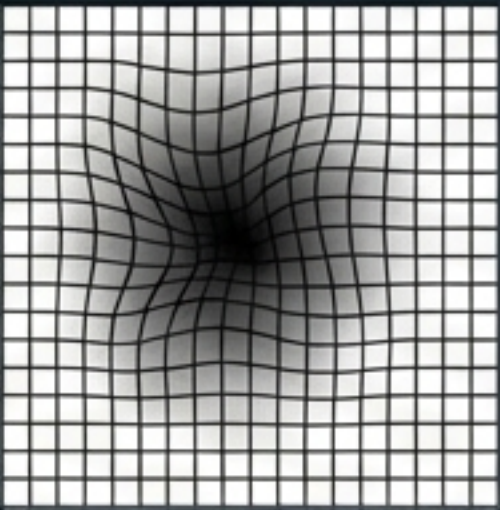
SYMPTOMS



DIAGNOSTICS



Normal Amsler Grid



Warped Amsler Grid (AMD)

Amsler grid testing

OCT
Macular exam

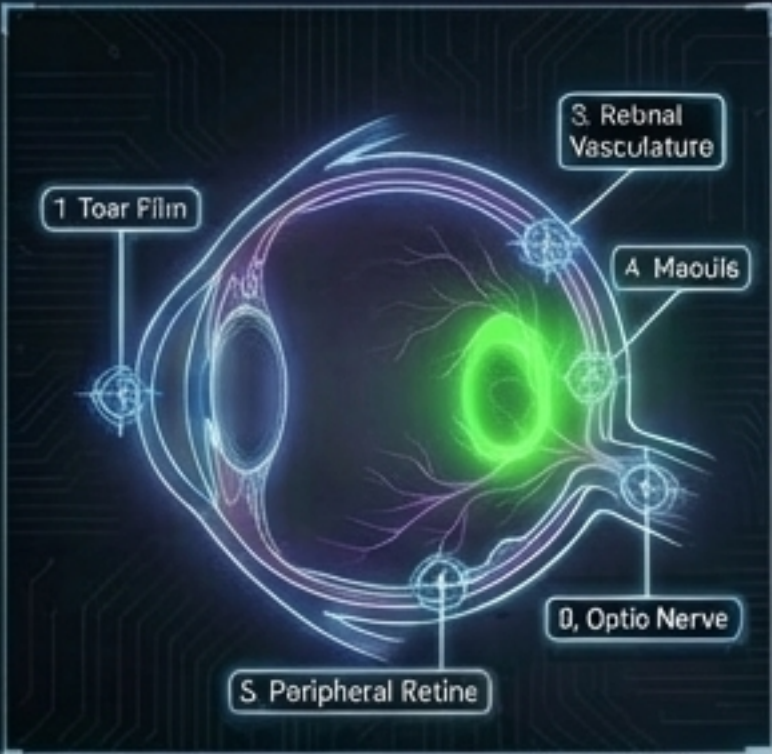
TREATMENTS



Anti-VEGF injections
(for Wet AMD)



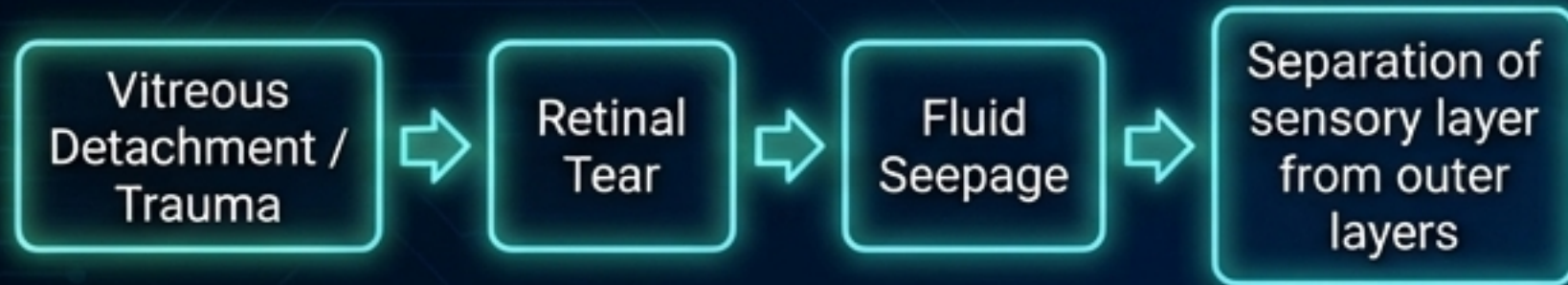
Targeted laser therapy



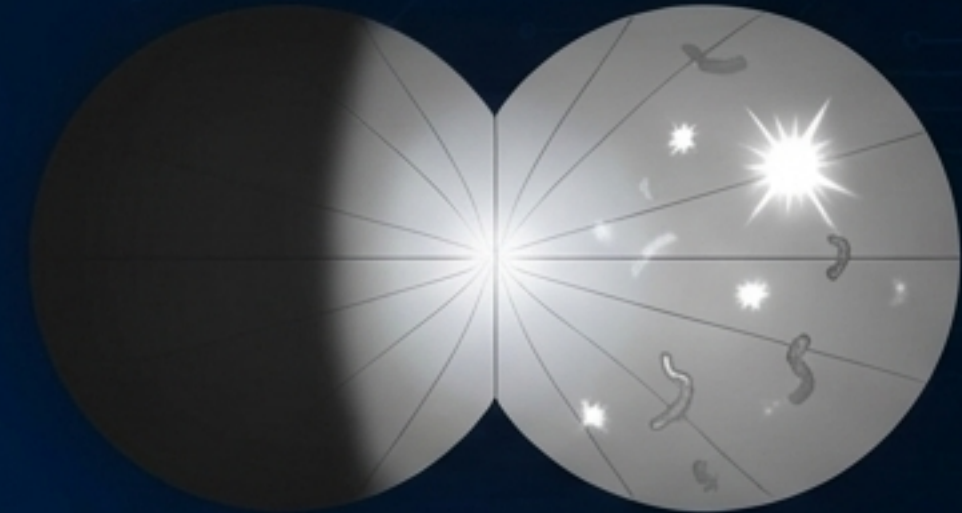
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DIAGNOSTICS REFERENCE MANUAL

CAUSES

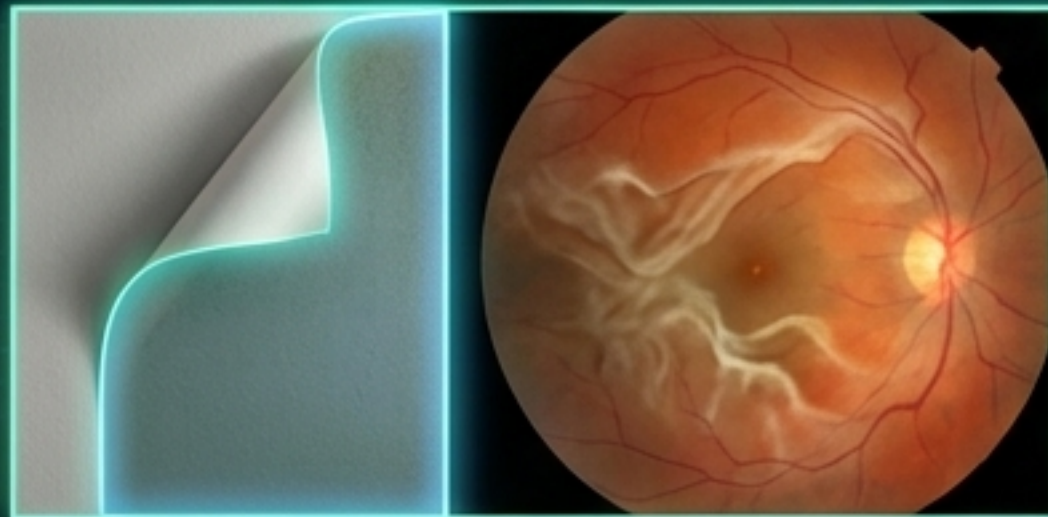


SYMPTOMS



Sudden bright flashes | Heavy shower of floaters | Dark curtain/visual field cut

DIAGNOSTICS



Binocular Indirect Ophthalmoscopy (BIO), Confrontation Visual Fields

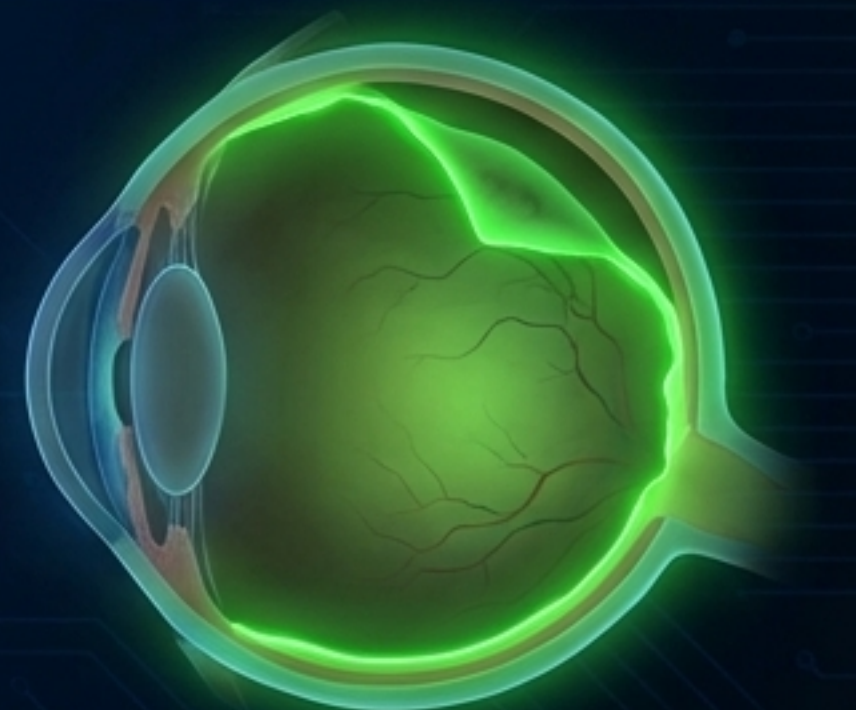
TREATMENTS



Retinal tear barricade



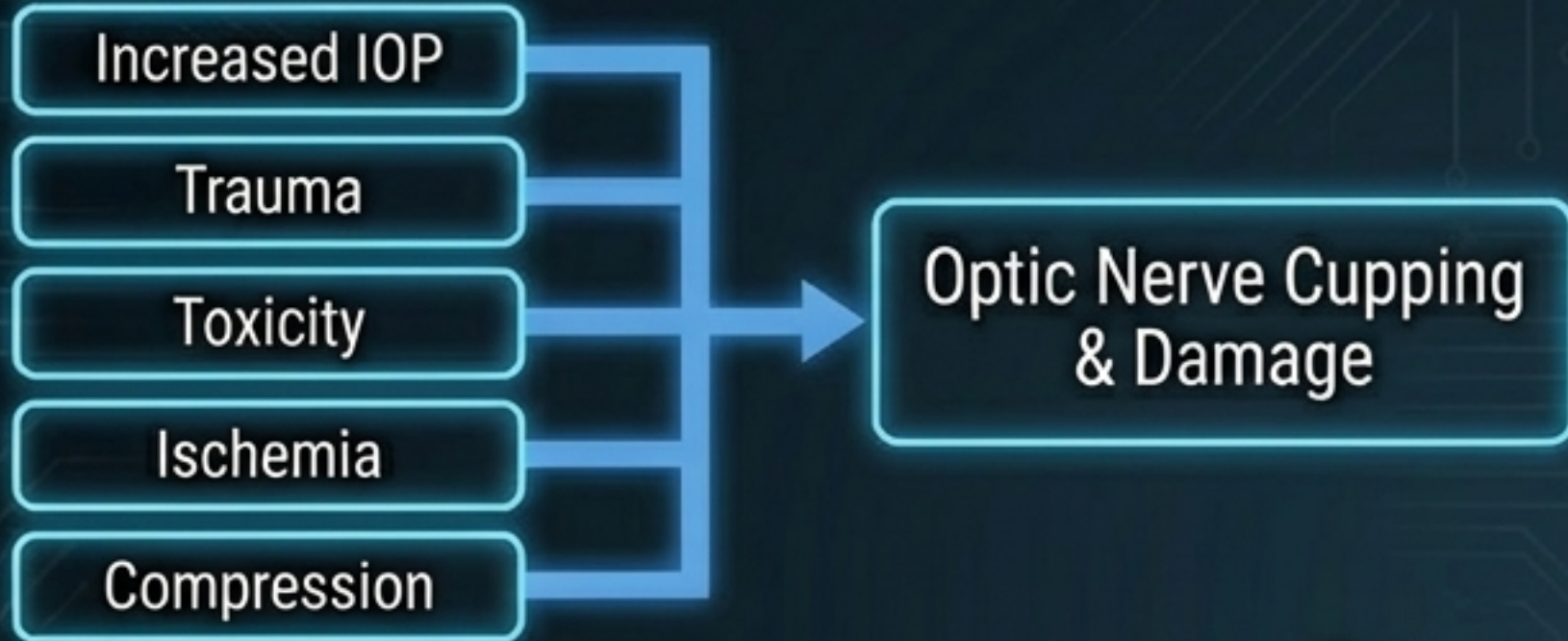
Pneumatic retinopexy, Scleral buckle



Anatomical Minimap

DIAGNOSTICS REFERENCE MANUAL

CAUSES



SYMPTOMS

Often asymptomatic until advanced stages.



Angle-Closure Variant: Sudden redness, severe pain, blur with halos, nausea.

DIAGNOSTICS

Tonometry
(Applanation/
Non-contact),
Automated
Perimetry, OCT



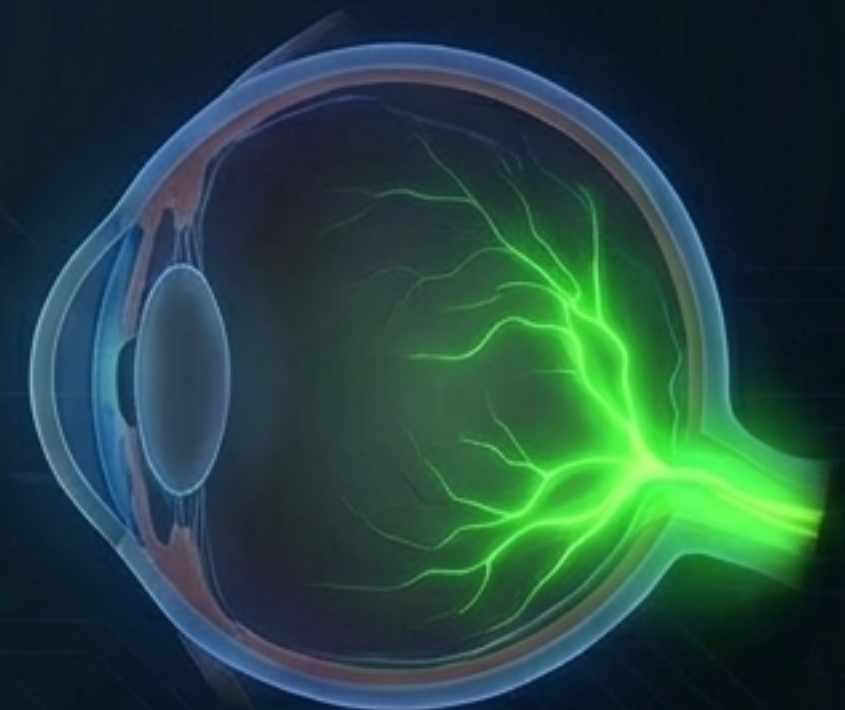
TREATMENTS



IOP-lowering topical medications



Selective Laser Trabeculoplasty (SLT)



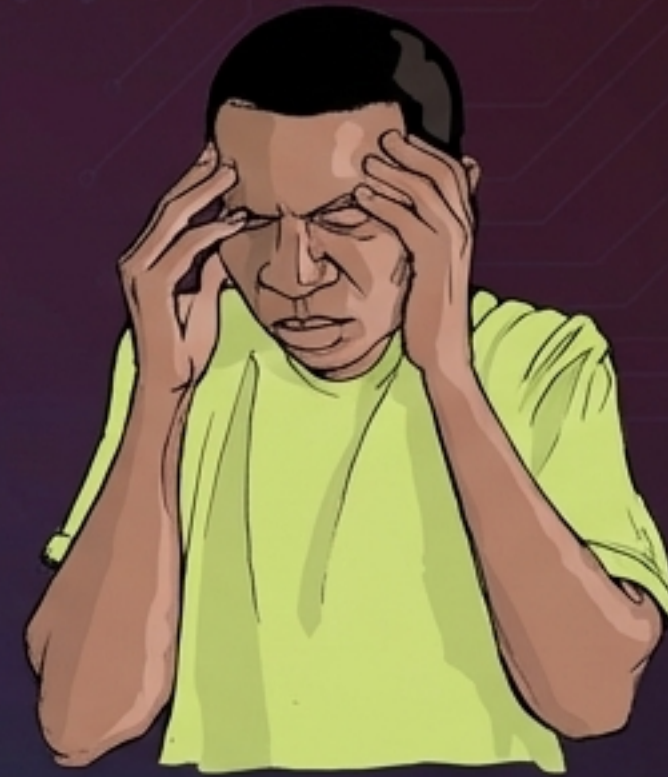
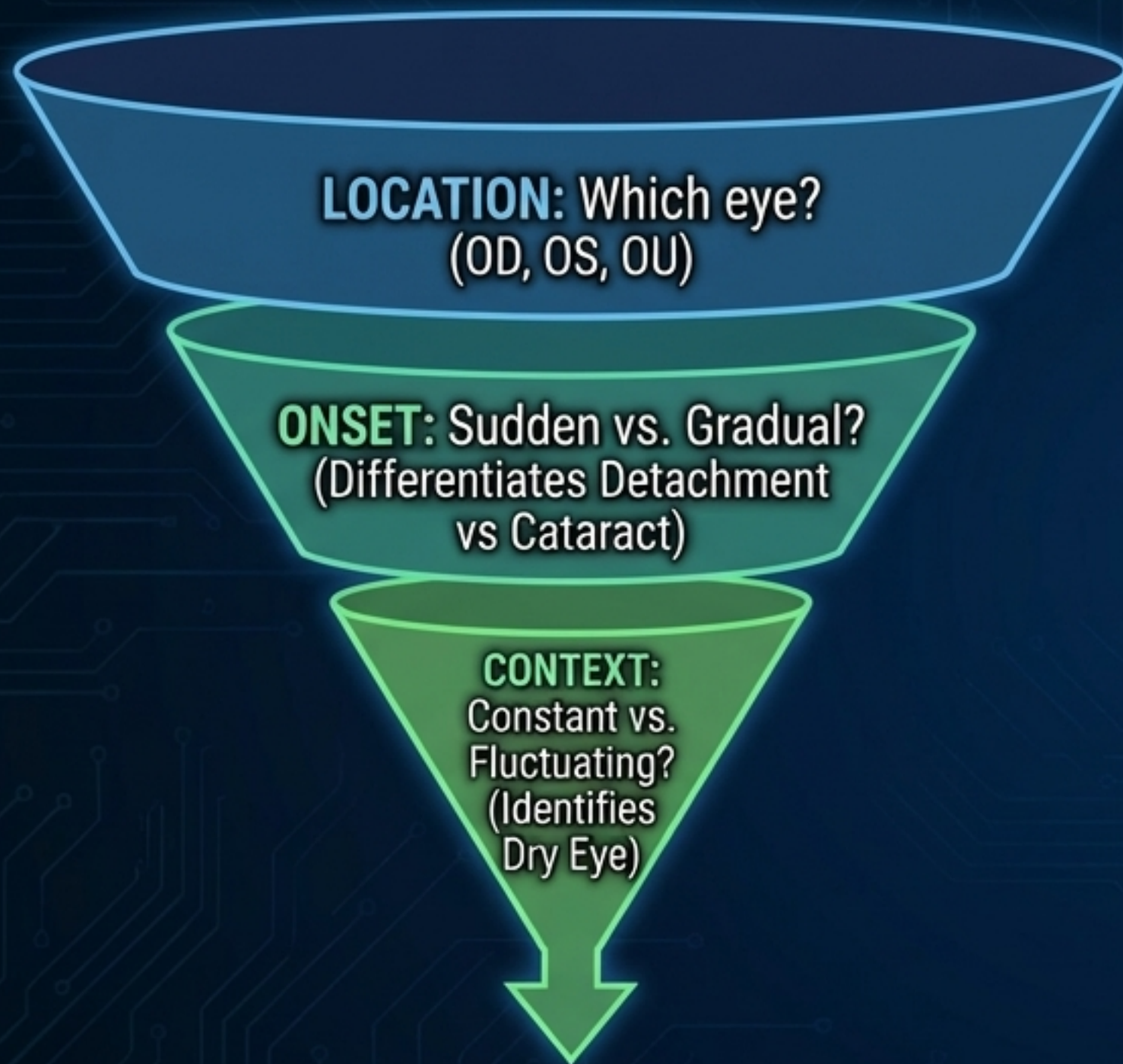
Anatomical Minimap

Diagnostic Differentiation Matrix

Disease	  Dry Eye	 Node (Fluctuating)	 Node (Gritty)					
	 Cataracts	 Node (Constant)						 Node
	 Diabetic Retinopathy	 Node (Patchy)				 Node (Cobwebs)		
	 Macular Degeneration	 Node (Wavy)		 Node				
	 Retinal Detachment				 Node (Curtain)	 Node (Sudden)	 Node	
	 Glaucoma		 Node (If Acute)		 Node (Tunnel)			
		Blurry Vision	Pain	Central Loss	Peripheral Loss	Floaters	Flashes	Glare
		Chief Complaints/Symptoms						

CLINICAL INSIGHT: A single symptom like "blurry vision" is merely a starting point. Diagnosis requires cross-referencing subjective symptoms with anatomical mechanics.

Distilling the Chief Complaint



IMMEDIATE ESCALATION TRIGGERS

- ⚠ 1. Sudden loss of vision (occurring in seconds or minutes)
- ⚠ 2. New flashes of light in the peripheral field
- ⚠ 3. Sudden, heavy shower of new floaters
- ⚠ 4. Severe, acute ocular pain with nausea

PROTOCOL: The technician's role is not to diagnose, but to translate the patient's subjective experience into objective anatomical mapping. Structure the history, recognize the red flags, and cue the correct diagnostic tests for the physician.